PT Eval IP Entered On: 4/29/2025 9:13 EDT
Performed On: 4/29/2025 8:48 EDT by Glotzbach, Brittany , PT

Assessment and Plan IP
Disposition Recommendations : Continued therapy flexing to patient tolerance
Discharge Equipment Needs : To Be Determined
Frequency : 3 Times a Week
Duration : 2
Duration Unit : Weeks
Therapeutic Procedures Interventions Planned : Therapeutic exercise, Neuromuscular Re-education, Gait training, Therapeutic Activities
Patient/Caregiver Goals : none stated
Plan for Next Visit : progress functional mobility as tolerated

Glotzbach, Brittany , PT - 4/29/2025 9:14 EDT
Impairments : ADL Deficits, Balance Deficit, Cognitive deficit, Endurance Decreased, Fall Risk, Gait Deficit, Generalized Weakness, Safety Awareness Deficit, Standing Tolerance Decreased, Transfer Deficit
Treating Diagnosis : Difficulty Walking, Weakness/Disuse Atrophy
Rehab Potential : Good

Glotzbach, Brittany , PT - 4/29/2025 9:12 EDT
Assessment/Rationale for Skilled PT : Pt is an 80 y.o. male presenting to BMH with UTI and confusion from Bethel Pointe where he was completeing rehab to home. Pt originally d/c'd to rehab on 4/6, came back on 4/9 with SBO/hernia repair and then discharged back to rehab where he has been since. Pt unable to recall what he has been working on at rehab since despite being A&Ox3 today. Prior to first admission at beginning of April, pt was living with his wife in a single story home where he was independent with toileting and gait with FWW. He req'd assist from wife for LB ADLs and getting out of bed at times. Wife does IADLs.
Today, pt req supervision for supine to sit and sitting balance at EOB. He req assist with socks this date (able to get the right foot off, but unable to put it on or do any of the left foot). Pt then req touchA for sit to stand and minA to take a couple steps with FWW to the chair. Pt with LOB posteriorly during transfer and demonstrates poor eccentric control when sitting. Pt continues to be below his baseline and will continue to benefit from skilled PT to address his deficits.
Barriers to safe discharge include: balance deficits, safety awareness deficits, cognitive deficits, and poor acitvity tolerance.

Glotzbach, Brittany , PT - 4/29/2025 9:14 EDT

History
Preferred Name : Steve
Referring Physician : Abbood, Hayder, MD
Patient Location : 5115
Medical Diagnosis : uti, confusion
Patient Precautions : Fall Precautions
Pertinent Medical History/Hospital Course : PMH: depression, HLD, HTN, GERD

Information Given By (Patient History) : Patient, Medical Record

Glotzbach, Brittany , PT - 4/29/2025 9:14 EDT

Prior Level of Function
Transfers : Independent
Bed Mobility : Independent
ADL's : Requires Assist
Ambulation : Independent

Glotzbach, Brittany , PT - 4/29/2025 9:14 EDT

Fall Comments : none
Mobility Level Prior to Admission : 7

Glotzbach, Brittany , PT - 4/29/2025 9:14 EDT